F&G ANNUITIES & LIFE

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# Field Impairment Underwriting Guide

This is a general rating guide for field underwriting only, reflecting our most commonly seen medical impairments. Not all situations are covered in this guide and with underwriting, results may vary based on age, date of diagnosis, and severity of condition.

Contact RiskAssessment@fglife.com for more assistance.

| Impairment | Rating Range |
|---|---|
| ADD/ADHD in adults no comorbid medical impairment | Standard |
| AIDS or ARC | Decline |
| Alcohol Use Disorder<br>Use within 2 years, No use > 2 years | Decline<br>Standard – Table 8 |
| Amyotrophic lateral sclerosis | Decline |
| Anemia – Iron Deficiency | Possibly Standard |
| Anemia – Other | Standard– Decline |
| Angina | See coronary artery disease |
| Ankylosing spondylitis | Standard – Table 4 |
| Anorexia nervosa | Standard – Decline |
| Anxiety | Standard – Table 3 |
| Aortic Aneurysm – Other | Decline |
| Aortic aneurysm – small, well followed | Table 3–4 |
| Aortic insufficiency | Standard – Table 8 |
| Arterial Vascular Dz | Table 4 – Decline |
| Arteriovenous malformations | Standard – Decline |
| Arthritis – osteoarthritis | Standard |
| Arthritis – rheumatoid or psoriatic | Standard – Table 6 |
| Ascites | Decline |
| Asthma | Standard – Table 4 |
| Atrial fibrillation or flutter | Table 3 – Decline |
| Atrial septal defect | Standard – Decline |
| Atrioventricular block 1st and 2nd | Standard |
| Atrioventricular block 3rd degree – complete | Table 4 – Decline |
| Basal cell carcinoma | Standard |
| Bells palsy | Standard |
| Benign prostate hypertrophy | Standard |
| Bicuspid aortic valve | Standard – Table 8 |
| Bigeminy | Standard – Table 8 |
| Bipolar Disorder | Table 3 – 8 |
| Bright's disease | Standard – Decline |
| Bronchiectasis | Table 3 – Decline |
| Bronchitis chronic | Table 4 – Decline |
| Bulimia | Standard – Decline |
| Bundle branch block – hemiblock or right | Standard – Table 3 |

| Impairment | Rating Range |
|---|---|
| Bundle branch block – left | Table 4 – Decline |
| Cancer – two years after treatment, no metastases, no recurrence | Standard – Decline |
| Cardiomyopathy | Table 3 to Decline |
| Carotid bruit | See Transient Ischemic Attack |
| Celiac disease | Standard – Table 4 |
| Cerebral palsy | Adult, gainfully employed & no assistance in walking Standard – Table 3 |
| Cerebral palsy – Juvenile | Contact HO – RiskAssessment@fglife.com |
| Cerebrovascular accident | See Stroke |
| Charcot Marie Tooth disease | Standard – Decline |
| Christmas disease (Factor IX deficiency) | Standard – Table 8 |
| Chronic Fatigue Syndrome | Standard – Table 3 |
| Chronic obstructive pulmonary disease (COPD) | Table 4 – Decline |
| Chronic pain – regular narcotic use | Table 3 – Decline |
| Chronic renal insufficiency / failure | Decline |
| Cirrhosis | Decline |
| Cocaine use – no use in past three years | $7.50/m – Decline |
| Colitis – not ulcerative & full recovery | Standard |
| Colon polyp(s) – benign | Standard |
| Congestive Heart Failure | Decline |
| Coronary artery disease | Within 3 months of surgery – PP<br>>3 months or controlled on medical therapy with no surgery planned<br>Age <40 – decline<br>Age 40–45 – T6 to decline<br>Age 46–59 – T4 to decline<br>Age >60 – T3 to decline<br>May be able to improve offer with favorable cholesterol and favorable follow up cardiac testing<br>Tobacco use within 12 months may mean higher rating or decline |
| Costochondritis | Standard |
| Crohn's disease | Standard – Decline Longer duration/milder disease may qualify for better rating |
| Cushing's syndrome | Standard – Table 4 |
| Cystic fibrosis | Decline |
| Cystitis | Standard |

| Impairment | Rating Range |
|---|---|
| Deep Venous Thrombosis | Two months after recovery<br>Standard |
| Defibrillator/Implanted | Decline |
| Dementia | Decline |
| Depression | Standard – Decline |
| Diabetes | Age <20 or Dialysis – decline<br><br>**Type 1 Diabetes**<br>Well controlled, no complications, and no tobacco use:<br>Age 20–29 – T8 to decline<br>Age 30 & up – T6 to T8<br><br>**Type 2 Diabetes**<br>Well controlled, no tobacco use:<br>Age 20–29 – T6 to decline<br>Age 30–49 – T3 to T6<br>Age 50 & up – Standard<br><br>**Gestational Diabetes**<br>Not currently pregnant and normal labs. Standard. |
| Diverticulitis/Diverticulosis – recovered | Standard |
| Down's syndrome | Decline |
| Drug abuse or addiction – no drug use in 3 years | Standard – Table 8 |
| Eating Disorders | Standard – Decline |
| Emphysema | Table 4 – Decline |
| Encephalitis – recovered no residuals | Standard |
| Endocarditis – recovered no residuals | Standard |
| Epilepsy | **Petit Mal**<br><6 months since diagnosis – decline<br><br>Well controlled, no seizures in last 2 years – standard – T3<br><br>Moderate control <7 seizures per year – T3 – T4<br><br>Poor control >6 seizures per year or prolonged unconsciousness or requiring multiple hospitalizations T4 – decline<br><br>**Grand Mal**<br><6 months since diagnosis – decline<br><br>Well controlled, no seizures in last 2 years – standard – T3<br><br>Moderate control, <7 seizures per year – T3 – T5<br><br>Poor control, >6 seizures per year or prolonged unconsciousness or requiring multiple medications – T6 – decline |

| Impairment | Rating Range |
|---|---|
| Esophageal varices | Decline |
| Factor VII or IX Deficiency | Table 4 – Decline |
| Fibrocystic breast disease | Standard |
| Fibromyalgia | Standard |
| Gallbladder (removed and B9) | Standard |
| Gastric bypass – > 1 after surgery | Standard – Table 3 |
| GERD | Standard |
| Gilbert's syndrome | Standard |
| Glomerulonephritis | Table 4 – Decline |
| Graves' disease – controlled | Standard |
| Guillain – Barre syndrome – recovered no residuals | Standard |
| Hashimoto's disease – controlled | Standard |
| Heart attack | Table 4 – Decline |
| Hematuria (resolved or no genitourinary disease) | Premenopausal Females – Standard<br>Males – Postpone until evaluated |
| Hemochromatosis – six months after dx well controlled | Standard – Table 4 |
| Hemophilia | Table 3 – Decline |
| Hemoptysis (unresolved or unexplained) | Decline |
| Hepatitis A or B | Standard to Decline. Better ratings dependent upon liver function and virology results. |
| Hepatitis C | HCV – RNA undetectable<br>Standard<br>Otherwise Decline |
| HIV–HIV–RNA counts undetectable | T8 – Decline |
| Hodgkin's disease – postpone one to five years depending on Stage | Standard – Decline |
| Huntington's chorea | Decline |
| Hydrocephalus | Table 3–8 |
| Hyperglycemia or Hypoglycemia – medically monitored and not diabetes | Standard |
| Hyperlipidemia – controlled | Standard |
| Hypertension – controlled | Standard |
| Hyperthyroidism or hypothyroidism | Standard |
| Hysterectomy | Standard unless due to Cancer |
| Idiopathic Hypertrophic Sub Aortic Stenosis | Table 4 – Decline |
| Ileitis | Standard – Table 8 |

| Impairment | Rating Range |
|---|---|
| Intermittent claudication | Table 3 – Decline |
| Irritable Bowel Syndrome | Standard |
| Kaposi's sarcoma | Decline |
| Kidney stones | Standard |
| Labyrinthitis | Standard |
| Left Anterior Hemiblock | Standard |
| Left Bundle Branch Block | Table 4 – Decline |
| Left Posterior Hemiblock | Standard |
| Legionnaire's Disease (full recovery) | Standard |
| Leukemia (five years after end of treatment) | Table 4 – Decline |
| Lupus (Discoid) > one year after diagnosis | Standard |
| Lupus (Systemic) | Table 3–8 |
| Lyme Disease (full recovery) | Standard |
| Lymphoma – postpone one to five years depending on stage | Standard – Decline |
| Marfan's Syndrome (no cardiac or vascular impairments) | Table 4 – Decline |
| Marijuana Use (infrequent and no criminal record) | Standard non–nicotine (if no use of nicotine for past 12 months) – more frequent use = higher rate |
| Melanoma (complete excision and established pathology) | Standard – Decline |
| Melena (unresolved or unexplained) | Decline |
| Meniere's Disease (controlled or | Standard |
| Meningitis (fully recovered without complications) | Standard |
| Migraines (fully evaluated) | Standard |
| Mitral insufficiency or regurgitation | Standard – Decline |
| Mitral Valve Prolapse | Standard |
| Monoclonal Gammopathy or MGUS (after two years stable labs) | Table 3 – Table 4 |
| Mononucleosis (recovered) | Standard |
| Multiple Sclerosis | Table 3 – Decline |
| Muscular Dystrophy | Standard – Decline |
| Myasthenia Gravis | Standard – Decline |
| Myelofibrosis | Decline |
| Myeloma | Decline |
| Myocardial Infarction | Table 4 – Decline |
| Myocarditis (single incident, one year full recovery) | Standard |

| Impairment | Rating Range |
|---|---|
| Myositis (chronic or progressive) | Decline |
| Narcolepsy | Standard – Table 4 |
| Nephritis | Standard – Decline |
| Neuritis | Standard – Table 3 |
| Neuropathy | Standard – Table 4 |
| Osteomyelitis (chronic stable, not progressive) | Table 3 – Table 4 |
| Osteoporosis | Standard |
| Pacemaker | Table 3 – Decline |
| Paget's Disease of Bone (not progressive) | Standard |
| Palpitations (normal cardiac workup) | Standard |
| Pancreatitis (chronic or recurrent) | Decline |
| Paraplegia | Table 6 – Decline |
| Parkinson's Disease | Table 3 – Decline |
| Patent Ductus Arteriosis – surgically corrected | Standard |
| Pericarditis (full recovery) | Standard |
| Peripheral Vascular Disease (non–smoker) | Standard – Table 4 |
| Phlebitis (full recovery) | Standard |
| Poliomyelitis (stable no wheelchair) | Standard – Decline |
| Polycystic Kidney Disease | Table 3– Decline |
| Polycythemia (diagnosed two years, stable CBC) | Table 3 – Table 6 |
| Polymyositis | Standard – Decline |
| Polyp (benign pathology) | Standard |
| Post–Traumatic Stress Disorder | Standard – Table 3 |
| Prostatitis (treated) | Standard |
| Proteinuria | Standard – Decline |
| Psoriasis | Standard |
| Psoriatic Arthritis | Standard – Table 6 |
| Pulmonary Embolism (after six months full recovery) | Standard – Table 4 |
| Pulmonary Fibrosis | Decline |
| Pulmonary Hypertension | Decline |
| Pyelonephritis (full recovery) | Standard |
| Quadriplegia (Tetraplegia) | Decline |
| Regional Enteritis | See Crohns Disease |
| Renal Dialysis | Decline |


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| Impairment | Rating Range |
|---|---|
| Renal Insufficiency / Failure | Decline |
| Renal Transplant | Table 6 – Decline |
| Restless Leg Syndrome | Standard |
| Rheumatic Fever (full recovery no cardiac residuals) | Standard |
| Rheumatoid Arthritis | Standard – Table 6 |
| Right Bundle Branch Block | Standard |
| Sarcoidosis (lungs/skin only and remission six months) | Standard |
| Schizophrenia | Decline |
| Scleroderma (skin only, two years after diagnosis) | Standard |
| Sclerosing Cholangitis | Decline |
| Sickle Cell Disease (normal CBC, no hospitalizations past five years) | Table 3 – Decline |
| Sickle cell trait | Standard |
| Sjogren's Syndrome | Standard |
| Sleep apnea (treated) | Standard – Table 3 |
| Spina Bifida (asymptomatic) | Standard |
| Stroke (after first year) | Table 4 – Decline |
| Suicide attempt (after first year) | Flat extra $3/M – Decline |
| Suicide attempts (>1attempt) two years after last | Table 4 – Decline |
| Tachycardia (cardiac workup neg) | Standard – Table 3 |
| Transient Ischemic Attack (after six months) | Table 3 –Table 6 |
| Traumatic Brain Injury (TBI) fully recovered > 6 months | Standard |
| Tremor (negative neurological evaluation) | Standard |
| Ulcerative Colitis | Table 3 – Table 8 |
| Urinary Tract Infection (UTI) | Standard unless recurrent |
| Vertigo (after neurological evaluation) | Standard |
| Wolff–Parkinson–White (WPW) | Standard |


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# Common reasons F&G orders medical records or declines to offer life coverage

F&G Underwriting is always looking for opportunities to assist you in determining which applications to send to us and to help you manage the expectations of your clients.

Below are some common reasons we may order medical records or deny coverage. The lists are not intended to be all–inclusive.

There are many other reasons we may order medical records or decline to issue coverage, but these are the ones we see most frequently.

## Common triggers requiring medical records

We will usually need an Attending Physician Statement (APS) if there is a reported history of serious medical concerns. Some common examples include:

- Undisclosed medical history
- Stroke
- Coronary artery disease, heart disease, heart attack or heart failure
- Drug or alcohol abuse
- Diabetes requiring insulin
- Cancer
- Pancreatic disease
- Liver disease or hepatitis
- Paralysis
- Seizures
- Psychological disorder *(other than mild anxiety or depression)*
- Respiratory diseases other than mild asthma
- Ulcerative colitis
- HIV *(Note: this would be highly rated if any offer is made)*
- Multiple sclerosis (MS)

## Common medical conditions that could lead to a decline

This is a list of issues we might find in someone's medical history that usually would result in a decline:

- Driving without a valid license
- Significant undisclosed medical history revealed by an FCRA source
- Use of drugs other than marijuana in the last two years
- Current prescription intended to help prevent drug or alcohol abuse or treatment for drug or alcohol overuse in the last two years
- Recurrent cancer (other than basal cell skin cancer)
- Any Stage IV cancers
- Uncontrolled diabetes or diabetes with significant complications
- Heart disease combined with a history of diabetes
- Heart disease combined with a history of cerebrovascular disease/stroke
- Awaiting a heart, lung or liver transplant, or a heart, lung or liver transplant recipient
- Treated with renal (kidney) dialysis
- Use of oxygen
- Currently hospitalized or residing in a care facility
- Stroke with severe complications
- Dementias; Alzheimer's disease
- Suicide attempt in the last two years or history of more than two suicide attempts if any in the last 10 years
- ALS
- Huntington's Chorea
- Liver cirrhosis; hepatitis D (Delta Hepatitis); biliary cholangitis or sclerosing cholangitis
- Psychosis with history of alcohol or drug abuse
- Developmental disorders in adults if unable to live independently
- AIDS (as a disorder, not HIV as an infection).
  *Note: We may be able to offer rated coverage for select individuals living with HIV.*
- Unexplained lab abnormalities *(we can often reconsider after medical evaluation at the applicant's expense)*


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## Other common reasons we may not offer coverage

While most declines are caused by medical issues, there are several other situations where F&G will not be able to issue coverage:

- Currently in prison, on probation or parole, or released from probation or parole in the last 12 months
- Conviction for serious crime(s) such as murder, rape, financial fraud, conspiracy, drug manufacture or delivery, or money laundering
- Unacceptable residency status in the United States
- Primary or all income received from outside the United States
- Inability to justify the total amount of coverage in force and applied
- Inability to obtain facultative reinsurance coverage when required
- High–risk activities
- Occupation involves the production, processing or sale of marijuana



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